

August 20, 2026

BlueCross BlueShield of South Carolina
Provider Appeals Department
P.O. Box 100300
Columbia, SC 29202

RE: Formal Appeal of Claim Denial — Jerrell Gerlach (MRN MUSC3906137), Claim MUSC-D48A-4

Patient	Jerrell Gerlach (DOB 1939-01-27)
MRN	MUSC3906137
Member ID / Group	BCB447669083 / GRP-66199
Claim number	MUSC-D48A-4
Date of service	2026-05-24
Procedure billed	CPT 93458 — Left heart catheterization with coronary angiography
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$12,559.35
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2026-10-12

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health respectfully submits this first-level provider reconsideration on behalf of patient Jerrell Gerlach (MRN: MUSC3906137, Member ID: BCB447669083, Group: GRP-66199) regarding the denial of Claim MUSC-D48A-4 for services rendered on May 24, 2026, at MUSC Health Rutledge Tower Ambulatory Care. BlueCross BlueShield of South Carolina denied the claim under CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") with RARC N115, citing a Local Coverage Determination, and remarked that "documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." We respectfully disagree and request full reversal and payment at the allowed amount of \$8,701.66.

CLINICAL BACKGROUND

Mr. Gerlach is an 87-year-old male with a longstanding and clinically complex cardiac history. He carries an active diagnosis of atrial fibrillation with onset dating to May 1994, managed continuously with Verapamil Hydrochloride, Digoxin, and Warfarin Sodium — a regimen in place for over three decades. His cardiac history includes multiple electrical cardioversions (July 2018, November 2019, November 2020) and two catheter ablations of cardiac tissue (July 2017, November 2018), reflecting a pattern of recurrent, refractory atrial fibrillation requiring escalating intervention. Concurrent active conditions include anemia (D64.9), prostate neoplasm under active treatment with Docetaxel and Leuprolide Acetate, and prediabetes. His former smoking status and lipid profile further contribute to his overall cardiovascular risk burden.

BASIS FOR APPEAL

The payer's denial appears to rest entirely on the primary diagnosis code submitted — J06.9 (Acute upper respiratory infection, unspecified) — which, standing alone, would not ordinarily support left heart catheterization with coronary angiography (CPT 93458). We acknowledge this coding discrepancy and submit that it does not accurately reflect the clinical indication for the procedure. The secondary diagnosis of D64.9 (Anemia, unspecified) was submitted alongside the claim, and the complete medical record documents active, long-standing atrial fibrillation as the patient's dominant cardiac condition. The presence of J06.9 as the primary billed diagnosis appears to be a coding error that does not reflect the treating physician's documented clinical rationale.

Dr. Elena V. Castellanos, MD (NPI 1881726354), performed CPT 93458 in the context of a patient with decades of atrial fibrillation, multiple failed cardioversions, prior catheter ablations, and ongoing antiarrhythmic and anticoagulation therapy. In an 87-year-old patient with this history, coronary angiography in conjunction with left heart catheterization is consistent with generally accepted standards of care for evaluating coronary anatomy prior to further electrophysiologic or structural intervention, or for assessing ischemic contributors to arrhythmia burden. The medical necessity of this procedure is supported by the totality of the patient's documented cardiac history, not by the primary diagnosis code alone.

We are submitting the complete encounter documentation, the remittance advice for Claim MUSC-D48A-4, and the relevant portions of Mr. Gerlach's longitudinal cardiac record in accordance with BlueCross BlueShield of South Carolina's reconsideration requirements through the My Insurance Manager portal. We respectfully request that the clinical reviewer evaluate medical necessity based on the full clinical record rather than the face of the submitted diagnosis code.

REQUESTED ACTION

MUSC Health requests that BlueCross BlueShield of South Carolina overturn the medical necessity denial for Claim MUSC-D48A-4, CPT 93458, date of service May 24, 2026, and reprocess the claim for payment at the previously established allowed amount of \$8,701.66. Should the reviewer determine that a coding correction is required to align the primary diagnosis with the documented clinical indication, we further request that the claim be reopened for resubmission with corrected diagnosis coding rather than adjudicated as a final denial. This reconsideration is submitted in advance of the appeal deadline of October 12, 2026.

Respectfully submitted,

Angela R. Whitfield, RN, BSN, CPC

Senior Appeals Specialist, Revenue Cycle Management
MUSC Health — Revenue Cycle / Patient Financial Services
(843) 792-2300 | muschealth.org

Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record